DESERT ORTHOPAEDIC CENTER

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Andrew B. Kim, D.O.
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Main Office	Northwest Office	Southwest Office	Henderson Office
2800 East Desert Inn Road Suite 100 Las Vegas, NV 89121	8402 West Centennial Parkway Las Vegas, NV 89149	8205 West Warm Springs Road Suite 250 Las Vegas, NV 89113	2930 West Horizon Ridge Parkway Suite 100 Henderson, NV 89052

FOR: Thomas Baker DOB:

ADDRESS: DATE:

Rx -in-home PT tx and eval
2 times a week x 2
1 skilled nursing visit for wound check

ICD 10: S/P L TKA

Refill: PRN: NR:

Refill: Times Dr.: R. Nevins. MD

Dr. License No.: DISPENSE ONLY AS WRITTEN ☐ Rx

DOC-70 (Rev. 1/18) DEA No.:

Abbotts: (702) 456-8099 11/17 White (Patient) Yellow (Office)

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Henderson
2930 West Horizon Ridge
Parkway, Suite 100
Henderson, NV 89052

Phone: (702) 731-1616 | Appointments: (702) 731-4088 | Fax: (702) 734-4900 | www.dodv.com

Desert Orthopaedic Center

Desert Inn

Appointment: 03/24/2026 10:40 AM

Provider: Nevins, Russell MD

Patient Information

Name:	Thomas Earl Baker	**Home Phone:**	
D.O.B:	01/29/1955	**Work Phone:**	
Sex:	Male	**Mobile Phone:**	(310) 961-6900
SSN:		**Email:**	baker101@aol.com
MRN:	MM0000059703	**Preferred Contact Method:**	Phone
PMS ID:	114700PAT000060216	**Language:**	English
Marital Status:	Single	**Emergency Contact:**	Karen Gibbs
Race:	White	**Emergency Contact Phone:**	7608615711
Ethnicity:	Not Hispanic or Latino	**Employer:**	
Address:	322 Karen Ave Unit 1203 Las Vegas, NV 89109	**Primary Care Provider:**	Moorhead, John MD
		Referring Providers:	Moorhead, John MD Nevins, Russell MD Huff, Lawrence Jr Song, Walter MD

Primary Insurance Information

Carrier:	Medicare of Nevada	**Address:**	PO Box 6770 Fargo, ND 58108
Policy #:	3CN5R10MG83		
Group ID/Name:		**Phone Number:**	(855) 609-9960

Secondary Insurance Information

Carrier:	Anthem Blue Cross Blue Shield PPO/EPO	**Address:**	PO Box 60007 Los Angeles, CA 900600007
Policy #:	JQE313W04356		
Group ID/Name:	CASUPWP0	**Phone Number:**	(877) 833-5742

Pharmacy Information

Name:	CVS/pharmacy #6867		
Phone Number:	7026967126	**Address:**	4490 PARADISE RD LAS VEGAS, NV 89169
Fax Number:	7026967369		

DESERT ORTHOPAEDIC CENTER
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Baker, Thomas

Visit Note - March 10, 2026

PMS ID:	Sex:	DOB:	Phone:	MRN:
114700PAT000060216	Male	01/29/1955	(310) 961-6900	MM0000059703

Allergies
Reviewed March 11, 2026.
No known drug allergies

Medications
Reviewed and no changes noted March 11, 2026.
amlodipine 10 mg Oral - tablet
atorvastatin 40 mg Oral - tablet
diclofenac sodium 75 mg Oral - tablet, delayed release (enteric coated)
gabapentin 800 mg Oral - tablet
trazodone 100 mg Oral - tablet

Medical History
Reviewed and no changes noted March 11, 2026.
Essential hypertension
Hyperlipidemia

Musculoskeletal History
Reviewed and no changes noted March 11, 2026.

Musculoskeletal Surgery
History of arthroscopy of knee joint: 30 years

Social History
Reviewed and no changes noted March 11, 2026.
EtOH none
Smoking status - Never smoker

Pneumonia vaccination not administered or previously received

Advance Care
Healthcare Proxy: No
Living Will: No

Chief Complaints:
1. F/U Knee Arthritis, Primary, Left evaluated on September 30, 2025

HPI: This is a 71 year old male who:
1. is following up for Knee Arthritis, Primary on the left knee joint. He was seen on September 30, 2025, at which time he was treated with Viscosupplementation - Orthovisc.

   The patient presents for status check.

Vitals:

Date	Taken By	B.P.	Pulse	Resp.	O2 Sat.	Temp.	Ht.	Wt.	BMI	BSA
03/10/26 13:07	Ferguson, Cheyenne	131/74 SIT					72.0 in	175.0 lbs	23.7	2
	FiO2									

* Patient Reported

Exam:

Knee

Right Knee ROM: **with pain**

Skin:
Right Knee: skin intact, no rashes or lesions.

Inspection:
Right Knee: **mild knee effusion, medial joint line tenderness, and varus alignment**

Right Quadriceps: Strength: 5/5, normal muscle tone.

Right Hamstring: Strength: 5/5, normal muscle tone.

Stability:
Right Knee: **mild pseudolaxity, medial compartment**

Special:
Right Knee: **Patellar Grind Test: positive**

Right patella tracking: **normal and crepitus 1**

Additional Notes: DNVI

Left Knee ROM: **with pain**

Skin:
Left Knee: **Old well healed open meniscectomy incision**

Inspection:
Left Knee: **mild knee effusion, medial joint line tenderness, and varus alignment**

Left Quadriceps: Strength: 5/5, normal muscle tone.

Left Hamstring: Strength: 5/5, normal muscle tone.

Stability:
Left Knee: **mild pseudolaxity, medial compartment**

Special:
Left Knee: **Patellar Grind Test: positive**

Left patella tracking: **normal and crepitus 1**

Impression/Plan:
1. Knee Osteoarthritis, Left
   Unilateral primary osteoarthritis, left knee (M17.12)

   Plan: Counseling - Knee DJD.

Viet A Nguyen, PA-C (Primary Provider)
Russell T Nevins, MD (Bill Under)
(702) 731-1616 Work
(702) 734-4900 Fax

Musculoskeletal care: I advised the patient that most patients with degenerative joint disease of the knee can achieve some measure of pain relief from nonsurgical options like medications, lifestyle modification, exercise and physical therapy, and use of supportive devices or change in footwear. The importance of maintaining an ideal body weight or losing weight to reduce stress on the knee joint is a cornestone of nonoperative management.

Medication Counseling
NSAIDS : I discussed with the patient that NSAIDs should be taken with food. Prolonged use of NSAIDs can result in the development of stomach ulcers or bleeding. Patient advised to stop taking NSAIDs if abdominal pain occurs. The patient verbalized understanding of the proper use and possible adverse effects of NSAIDs. All of the patient's questions and concerns were addressed.

OTC Recommendations:
NSAIDs: Discussed Meloxicam for pain. Take with food. If medication helps he will speak with his PCP about long term use.

Active Therapy : Active therapies include stretching exercises, weight training (quadriceps strengthening), and cardiovascular non-impact conditioning. Weight reduction may be emphasized as appropriate.
Passive Therapy : Passive therapies include ice, heat, massage, ultrasound, and electrical stimulation. Your therapist will decide which of these interventions will be most beneficial to you.

Surgical Options and Alternatives
Intraarticular steroid injection : I discussed with the patient that this involves the injection of a potent antiinflammatory substance (cortisone) into the knee joint. Risks include whitening of the skin at the injection site and a transient rise in blood glucose. Complications are extremely rare; infection is the most common. There is no rule as to how many injections can be given. Because some research has shown that too much cortisone can damage cartilage, most physicians limit the number of shots they will give you.
Intraarticular viscosupplementation : I discussed with the patient that this procedure involves the injection of artificial joint fluid into the knee. The fluid acts as a lubricant, shock absorber, and an antiinflammatory agent. Depending on the specific agent used you will receive one to five shots, each shot one week apart. This intervention is most effective if used in early stage osteoarthritis. Side effects include knee swelling and pain at the injection site. Long term efficacy is not currently known. Complications include but are not limited to infection and joint swelling.
Total knee replacement : I explained to the patient that this is an option for patients that have severe pain in their knee from arthritis who have failed conservative management. The diseased parts of the knee are replaced with metal and plastic. Approximately 90% of patients who have total knee replacement experience a significant reduction in pain and a dramatic improvement in their ability to perform common activities of daily living. High impact activities should be avoided after surgery, if possible, to prolong the life of the implant. Serious complications occur in less than 2% of patients. Risks include but are not limited to infection, injury to blood vessels and nerves, blood clots, heart complications, lung complications, stroke, and stiffness.

Diagnostic Imaging Results
I reviewed and discussed the results of the X-Ray(s) with the patient.

After counseling the patient, we decided on the following plan for the LEFT KNEE: Total knee replacement

2. **Knee Osteoarthritis, Right**
Unilateral primary osteoarthritis, right knee (M17.11)

Plan: Counseling - Knee DJD.
Musculoskeletal care: I advised the patient that most patients with degenerative joint disease of the knee can achieve some measure of pain relief from nonsurgical options like medications, lifestyle modification, exercise and physical therapy, and use of supportive devices or change in footwear. The importance of maintaining an ideal body weight or losing weight to reduce stress on the knee joint is a cornestone of nonoperative management.

Medication Counseling

NSAIDS : I discussed with the patient that NSAIDs should be taken with food. Prolonged use of NSAIDs can result in the development of stomach ulcers or bleeding. Patient advised to stop taking NSAIDs if abdominal pain occurs. The patient verbalized understanding of the proper use and possible adverse effects of NSAIDs. All of the patient's questions and concerns were addressed.

OTC Recommendations:
NSAIDs: Discussed Meloxicam for pain. Take with food. If medication helps he will speak with his PCP about long term use.

Active Therapy : Active therapies include stretching exercises, weight training (quadriceps strengthening), and cardiovascular non-impact conditioning. Weight reduction may be emphasized as appropriate.
Passive Therapy : Passive therapies include ice, heat, massage, ultrasound, and electrical stimulation. Your therapist will decide which of these interventions will be most beneficial to you.

Surgical Options and Alternatives
Intraarticular steroid injection : I discussed with the patient that this involves the injection of a potent antiinflammatory substance (cortisone) into the knee joint. Risks include whitening of the skin at the injection site and a transient rise in blood glucose. Complications are extremely rare; infection is the most common. There is no rule as to how many injections can be given. Because some research has shown that too much cortisone can damage cartilage, most physicians limit the number of shots they will give you.
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Total knee replacement : I explained to the patient that this is an option for patients that have severe pain in their knee from arthritis who have failed conservative management. The diseased parts of the knee are replaced with metal and plastic. Approximately 90% of patients who have total knee replacement experience a significant reduction in pain and a dramatic improvement in their ability to perform common activities of daily living. High impact activities should be avoided after surgery, if possible, to prolong the life of the implant. Serious complications occur in less than 2% of patients. Risks include but are not limited to infection, injury to blood vessels and nerves, blood clots, heart complications, lung complications, stroke, and stiffness.

Diagnostic Imaging Results
I reviewed and discussed the results of the X-Ray(s) with the patient.

Follow up PRN for: Further Evaluation and Management - (with Physician). Other Instructions: will schedule for pre-op appt once medically cleared, with DR. Nevins

Note:

Patient presents for bilateral knee follow up. He has end stage arthritis of bilateral knees. He would like to proceed with total knee replacement.

I discussed with him that given his current minimal pain, the goal of the surgery should be to give him more functionality in his knees. I explained to him that there are no guarantees with total knee replacement, and that he may have more pain following knee replacement which he fully understands. His biggest complaint with his knees is that during periods of standing that it becomes difficulty to move again and issues going up and down stairs.

Understanding the risks, we will proceed with total knee replacement. He will follow up with Dr. Nevins for pre-operative evaluation.

Staff:

Viet A Nguyen, PA-C (Primary Provider)

Cheyenne Ferguson

Nevins, Russell MD (Bill Under)

Agree with above.

Knee Osteoarthritis, Left: .

Plan: Counseling - Knee DJD.

Knee Osteoarthritis, Right: .

Plan: Counseling - Knee DJD.

Electronically Signed By: Russell T Nevins, MD, 03/18/2026 06:39 AM PDT

I saw the patient with Viet Nguyen, elicited a history, performed an exam, and developed an impression and plan. I was present for all key aspects of the visit. My impression is as follows:

Knee Osteoarthritis, Left: .

Plan: Counseling - Knee DJD.

Knee Osteoarthritis, Right: .

Plan: Counseling - Knee DJD.

Electronically Signed By: Viet A Nguyen, PA-C, 03/17/2026 08:01 AM PDT